

Patient Demographics

Patient Demographics				
Name	Patient ID	SSN	Gender Identity	Birth Date
Powell, Donna Rosemary	23707922	xxx-xx-1508	Female	07/20/62 (63 yrs)
Address	Phone	Email		
71 Dick Rd Great Falls MT 59404	303-802-9404 (M)	—		
Reg Status	PCP	Date Last Verified	Next Review Date	
Verified Registration	William Jacob Joshu, PA	07/10/26	01/06/27	

Powell, Donna Rosemary #23707922
(Acct:8209347668) (63 year old F) PCP: JOSHU, W

IR

ED Arrival Information

ED Arrival Information			
Expected	Arrival	Acuity	
-	7/10/2026 14:59	Level 3	
Means of arrival	Escorted by	Service	Admission type
Private Transportation	Self	Emergency	Emergency
Arrival complaint			
Sent for CT			

Chief Complaint

Complaint	Comment
Back Pain	

OB/Gyn Status

LMP:	Unknown
------	---------

ED Treatment Team

Provider	Role	From	To	Phone	Pager
Kelly B Colleton, PA-C	Attending Provider	07/10/26 1549	--	775-445-8000	--
Emma Downs, RN	Registered Nurse	07/10/26 1551	--	--	--

ED Provider Notes**ED Provider Notes by Kelly B Colleton, PA-C**

07/10/26 1549

Author: Kelly B Colleton, PA-C Service: Emergency Author Type: Physician Assistant
Filed: 07/10/26 1812 Date of Service: 07/10/26 1549 Status: Signed
Editor: Kelly B Colleton, PA-C (Physician Assistant)

EMERGENCY MEDICINE NOTE:**DATE & TIME OF SERVICE:** 07/10/26**PATIENT:** Donna Rosemary Powell; 63 year old female**CHIEF COMPLAINT:**^[KC.1] Back Pain^[KC.2]**Means of Arrival:**^[KC.1] Private Transportation^[KC.2]**Final Diagnoses:**^[KC.1]

	ICD-10-CM
1. Acute left-sided low back pain with left-sided sciatica	M54.42 ^[KC.]

ED Provider Notes (continued)

ED Provider Notes by Kelly B Colleton, PA-C (continued)

07/10/26 1549

2]

HISTORY OF PRESENT ILLNESS:

63 year old female presents with complaints of left-sided flank and low back pain for more than 1 week. She reports that previous to this she was driving back from Montana on the car for 2 days. She has had some pain somewhat similar to this but it usually resolved with Aleve or Tylenol. She states it is primarily in the low buttocks region and does somewhat travel to the anterior aspect of her thigh. She denies any true radiating or zapping sensation down her leg. She denies any weakness or paresthesias. No saddle anesthesia. No loss of bowel or bladder control. Denies any abdominal pain nausea or vomiting diarrhea or dark or tarry stool. She denies any urinary symptoms such as dysuria, increased frequency or urgency.

PAST MEDICAL HISTORY:^[KC.1]History reviewed. No pertinent past medical history.^[KC.2]**MEDICATIONS & ALLERGIES:**

Medications:

Current Medications^[KC.1]

No current facility-administered medications for this encounter.

Current Outpatient Medications:

- cholecalciferol 2000 units tablet, Take 2,000 Units by mouth once daily., Disp: , Rfl:
- cyclobenzaprine (FLEXERIL) 10 mg tablet, Take 10 mg by mouth., Disp: , Rfl:
- hydrocortisone 2.5 % cream, , Disp: , Rfl:
- Mag Chloride-Calcium Carb (SLOW-MAG) 64 mg elemental, Take 64 mg by mouth once daily., Disp: , Rfl:
- methocarbamol (ROBAXIN) 500 mg tablet, , Disp: , Rfl:
- methylPREDNISolone (MEDROL, PAK,) 4 MG therapy pak tablet, follow package directions, Disp: 21 tablet, Rfl: 0
- Multiple Vitamins-Minerals (MULTI FOR HER) tablet, Take 1 tablet by mouth., Disp: , Rfl:
- niacin 100 mg tablet, Take 100 mg by mouth 2 times a day., Disp: , Rfl:

Allergies

Allergen

Reactions

- Warfarin

Rash^[KC.2]**PAST SURGICAL HISTORY:**^[KC.1]No past surgical history on file.^[KC.2]**FAMILY HISTORY:**^[KC.1]No family history on file.^[KC.2]**SOCIAL HISTORY:**

Social History

Social History**Tobacco Use**

- Smoking status: Never
- Smokeless tobacco: Never

Substance Use Topics

- Alcohol use: Not Currently
Comment: socially
- Drug use: Never^[KC.1]

PCP:^[KC.1] William Joshu^[KC.2]**PHYSICAL EXAM:**^[KC.1]

ED Provider Notes (continued)**ED Provider Notes by Kelly B Colleton, PA-C (continued)**

07/10/26 1549

Vitals

	07/10/26 1520	07/10/26 1522	07/10/26 1730
BP:	127/66		120/66
Pulse:	94		59
Temp:	35.4 °C (95.8 °F)		35.9 °C (96.6 °F)
Resp:	16		16
Weight:		64.8 kg (142 lb 13.7 oz)	
SpO2:	91%		95% ^[KC2]

Physical Exam

Vitals reviewed.

Constitutional:

General: She is not in acute distress.

Appearance: Normal appearance. She is not diaphoretic.

HENT:

Head: Normocephalic and atraumatic.

Eyes:

Conjunctiva/sclera: Conjunctivae normal.

Pupils: Pupils are equal, round, and reactive to light.

Cardiovascular:

Rate and Rhythm: Normal rate and regular rhythm.

Pulses: Normal pulses.

Radial pulses are **2+** on the right side and **2+** on the left side.

Heart sounds: Normal heart sounds, S1 normal and S2 normal.

Pulmonary:

Effort: Pulmonary effort is normal. No respiratory distress.

Breath sounds: Normal breath sounds. No stridor. No wheezing, rhonchi or rales.

Abdominal:

General: There is no distension.

Palpations: Abdomen is soft.

Tenderness: There is no abdominal tenderness. There is no right CVA tenderness, left CVA tenderness, guarding or rebound.

Musculoskeletal:Lumbar back: **Tenderness** present. No swelling, edema, deformity, signs of trauma, spasms or bony tenderness. Normal range of motion. **Positive left straight leg raise test**. Negative right straight leg raise test. No scoliosis.

Right lower leg: No edema.

Left lower leg: No edema.

Skin:Capillary Refill: Capillary refill takes **2 to 3 seconds**.**Neurological:**

General: No focal deficit present.

Mental Status: She is alert and oriented to person, place, and time.

LABS:^[KC1]

ED Provider Notes (continued)**ED Provider Notes by Kelly B Colleton, PA-C (continued)**

07/10/26 1549

Recent Results (from the past 24 hours)**POCT urinalysis device results**

Collection Time: 07/10/26 12:38 PM

Result	Value	Ref Range
Color, Urine - External	Yellow	Yellow
Appearance, Urine - External	Clear	Clear
Glucose, Urine - External	Negative	Negative
Bilirubin, Urine - External	Negative	Negative
Ketones, Urine - External	Negative	Negative
Specific Gravity, Urine - External	1.020	1.005 - 1.030
Blood, Urine - External	Trace-intact (A)	Negative
pH, Urine - External	7.0	5.0 - 8.0
Protein, Urine - External	Negative	Negative
Urobilinogen, Urine - External	0.2	<1.0 E.U./dL
Nitrites, Urine - External	Negative	Negative
Leukocyte Esterase, Urine - External	Negative	Negative

CBC With Plt Count And Auto Diff

Collection Time: 07/10/26 3:58 PM

Result	Value	Ref Range
WBC Count	6.0	3.7 - 10.6 x10E3/uL
RBC	4.39	4.19 - 5.21 x10e6/uL
Hemoglobin	14.0	12.3 - 16.0 g/dL
Hematocrit	41.1	36.0 - 47.0 %
Mean Corpuscular Volume	93.6	81.0 - 99.0 fL
Mean Corpuscular Hemoglobin	31.8	28.0 - 33.8 pg
Mean Corpuscular HGB Concentration	34.0	33.1 - 36.5 g/dL
Red Cell Distribution Width	13.3	11.8 - 14.0 %
Platelet Count	272	146 - 390 x10E3/uL
Mean Platelet Volume	7.9	6.4 - 10.2 fL
Neutrophil %	59.3	41.7 - 82.3 %
Lymphocyte %	31.2	10.8 - 44.4 %
Monocyte %	7.2	5.0 - 12.8 %
Eosinophil %	1.3	0.0 - 6.6 %
Basophil %	1.0	0.0 - 1.3 %
Neutrophil #	3.60	1.40 - 8.00 x10E3/uL
Lymphocyte #	1.90	1.00 - 5.20 x10E3/uL
Monocyte #	0.40	0.16 - 1.00 x10E3/uL
Eosinophil #	0.10	0.00 - 0.80 x10E3/uL
Basophil #	0.10	0.00 - 0.30 x10E3/uL

Comprehensive Metabolic Panel

Collection Time: 07/10/26 3:58 PM

Result	Value	Ref Range
Sodium, Serum or Plasma	141	136 - 144 mmol/L
Potassium	3.7	3.6 - 5.1 mmol/L
Chloride	106	102 - 110 mmol/L
Carbon Dioxide	28	22 - 32 mmol/L
Alkaline Phosphatase	81	38 - 126 U/L
AST	19	15 - 41 U/L
ALT	16	14 - 54 U/L
BUN	18	8 - 20 mg/dL
Creatinine, Serum or Plasma	0.93	0.60 - 1.10 mg/dL
Calcium	9.1	8.7 - 10.3 mg/dL
Protein, Total, Serum or Plasma	6.4	6.4 - 8.2 g/dL
Albumin	4.5	3.5 - 4.8 g/dL
A/G Ratio	2.4 (H)	1.0 - 2.2 ratio

ED Provider Notes (continued)**ED Provider Notes by Kelly B Colleton, PA-C (continued)**

07/10/26 1549

Glomerular Filtration Rate (GFR)	61	>60 mL/min/1.7
Globulins, Total	1.9 (L)	2.6 - 3.1 g/dL
Glucose, Serum or Plasma	188 (H)	60 - 99 mg/dL
Bilirubin, Total	0.4	0.4 - 2.0 mg/dL
Anion Gap	7	2 - 11 mmol/L ^[KC.2]

IMAGING:^[KC.1]

CT Abdomen Pelvis WO IV Cont

Result Date: 7/10/2026

1. No hydronephrosis or nephroureterolithiasis. 2. No acute inflammatory mass, bowel obstruction, or ascites. 3. Distal colonic diverticulosis without evidence of diverticulitis. Electronically Signed by: Scott Thalman 7/10/2026 3:54 PM

XR LUMBAR SPINE 2 TO 3 VIEWS

Result Date: 7/10/2026

Lower lumbar facet degeneration.^[KC.2]**MEDICAL DECISION MAKING:**

63 year old female arrives with concerns of left-sided low back pain. Ongoing for more than 1 week.

On independent chart review patient has been evaluated for low back pain on the right side 2 different times in the past week. Initially evaluated at renown urgent care 07/06/2026. During that visit thought to be acute left-sided low back pain with left-sided sciatica given Toradol and Robaxin with some improvement in pain. Return to the renown urgent care earlier today given she continued to have low back pain discomfort. During this visit they obtained lumbar x-rays and UA. X-ray showed no acute fracture or dislocation. UA did show some microscopic hematuria. Has some concerns for possible kidney stone and advised she give further evaluation at the ED including a CT to evaluate for kidney stone. Did not want to go to renown or travel via EMS therefore brought herself via POV.

On initial exam patient is alert and in no acute distress. Vitals WNL. No CVAT. Abdomen soft no distention or TTP. Palpation of low back shows no midline tenderness, palpable step-offs or depressions. TTP on some reproduction of symptoms over SI joint. No pain over the greater trochanter. Positive straight leg raise on the left. Does have poor hip IR and ER ROM and mobility. Normal strength and reflexes. Limb is otherwise neurovascularly intact. With good distal pulses and cap refill. Physical exam otherwise unremarkable.

Obtained labs and CT renal to rule out kidney stone. Please see ED course for interpretation evaluation.

At this time patient presentation is most consistent with acute left-sided low back pain with sciatica. They had concerns for possible kidney stone but no evidence of stone on imaging or hydronephrosis. No secondary signs of pyelonephritis. On reexamination of her urine there was only trace blood it does not appear infectious so unlikely UTI. No other back pain red flags on physical or history. Presentation not consistent with malignancy, fracture, cauda equina, viscous perforation, osteomyelitis or epidural abscess. Patient has already been prescribed muscle relaxers and continues to have discomfort. Prescribed Medrol Dosepak and sent referral to physical therapy as this will be the most beneficial in attempting to treat her discomfort. She verbalized understanding of plan and strict return precautions. Stable at discharge.^[KC.1]

ED Course as of 07/10/26 1812

Fri Jul 10, 2026

1606 **CT Abdomen Pelvis WO IV Cont****IMPRESSION:**

1. No hydronephrosis or nephroureterolithiasis.
2. No acute inflammatory mass, bowel obstruction, or ascites.
3. Distal colonic diverticulosis without evidence of diverticulitis.

^[KC]

ED Provider Notes (continued)**ED Provider Notes by Kelly B Colleton, PA-C (continued)**

07/10/26 1549

1634 **WBC: 6.0** [KC]
1634 **HGB: 14.0** [KC]
1635 On independent review of the patient's UA from outside facility there was only trace amount of blood no evidence of any other infectious process such as bacteria, nitrites or leukocytes. [KC]
1649 **Sodium: 141** [KC]
1649 **Potassium: 3.7** [KC]
1649 **Chloride: 106** [KC]
1649 **Carbon Dioxide: 28** [KC]
1649 **Alkaline Phosphatase: 81** [KC]
1649 **AST: 19** [KC]
1649 **ALT: 16** [KC]
1649 **BUN: 18** [KC]
1649 **Creatinine: 0.93** [KC]
1649 **Calcium: 9.1** [KC]
1649 **Anion Gap: 7** [KC]
1649 **Bilirubin, Total: 0.4** [KC]
1649 **Glucose(!): 188** [KC]

ED Course User Index

[KC] Kelly B Colleton, PA-C

Clinical Impressions as of 07/10/26 1812Acute left-sided low back pain with left-sided sciatica^[KC.2]Source: History was primarily obtained from patient

External Medical Record Review: The following external records were independently reviewed: Discharge summary.

Relevant review notes: Urgent care notes from 7/sections 7/10.

Labs reviewed: I independently interpreted this patient's laboratory studies as documented in MDM or ED Course. And available for review in ED course.

Imaging: I independently interpreted the patient's imaging studies: Imaging interpretation documented in ED course.. And available for review in ED course

EKG: N/A

Consultations:

Case was discussed with the following providers: N/A.

Details of any discussion with consultants included in MDM.

Medications:^[KC.1]**Medication Administrations (24 hours)**

(last 24 hrs)

**** No medications to display ****^[KC.2]

If patient required parenteral (intravenous) medications, they are listed above.

The patient received the following medications which required intensive monitoring due to high risk of morbidity/mortality:
N/A**Mandatory Screening for select patients:**

Powell, Donna Rosemary (MRN 23707922) Printed by Kelly B Colleton, PA-C [151426] at 7/13/2026 7:12 PM MDT

ED Provider Notes (continued)**ED Provider Notes by Kelly B Colleton, PA-C (continued)**

07/10/26 1549

Sexually transmitted disease testing was offered in the emergency department: Not applicable
Syphilis screening was offered in the emergency department: not applicable as the patient was not pregnant

Additional ED course/events:

Procedures:

Procedures

FINAL DIAGNOSES:^[KC.1]

Final diagnoses:

Acute left-sided low back pain with left-sided sciatica^[KC.2]

Secondary Diagnoses: The patient is suffering from the following comorbid conditions which complicate their care and may put them at higher risk: N/A.

Social Determinants of Health: Additionally the patient's health is significantly affected by certain social determinants of health including: N/A

DISPOSITION:

Discharge: Counseled on appropriate follow up, discharge instructions, and return precautions. All questions answered.^[KC.1]

Discharge Medication List as of 7/10/2026 5:07 PM**START taking these medications**

	Details
methyIPREDNISolone (MEDROL, PAK,) 4 MG therapy pak tablet	follow package directionsDisp-21 tablet, R-0, Normal

CONTINUE these medications which have NOT CHANGED

	Details
cholecalciferol 2000 units tablet	Take 2,000 Units by mouth once daily.Historical Med
cyclobenzaprine (FLEXERIL) 10 mg tablet	Take 10 mg by mouth.Historical Med
hydrocortisone 2.5 % cream	Historical Med
Mag Chloride-Calcium Carb (SLOW-MAG) 64 mg elemental	Take 64 mg by mouth once daily.Historical Med
methocarbamol (ROBAXIN) 500 mg tablet	Historical Med
Multiple Vitamins-Minerals (MULTI FOR HER) tablet	Take 1 tablet by mouth.Historical Med
niacin 100 mg tablet	Take 100 mg by mouth 2 times a day.Historical Med ^[KC.2]

ED Provider Notes (continued)**ED Provider Notes by Kelly B Colleton, PA-C (continued)**

07/10/26 1549

Prescription management: This patient requires outpatient prescription drug management and has been prescribed medications listed above.

Controlled Substances: N/A^[KC.1]

Kelly Colleton, PA-C^[KC.2]

CTEP electronic signature

Portions of this document may have been created by nursing or other staff, these were reviewed and accepted by me. Appropriate corrections were made. This document was created using a computerized dictation system. Although I have reviewed this document, there may still be some phonetic and other typographical errors due to imperfections in the software program and should not be construed in any way to compromise the patient's medical care.

Kelly B Colleton, PA-C

07/10/26 1812

^[KC.2]

Attribution Key

KC.1 - Kelly B Colleton, PA-C on 07/10/26 1549

KC.2 - Kelly B Colleton, PA-C on 07/10/26 1812

ED Course

Value	Comment	By	Time
CT Abdomen Pelvis WO IV Cont	IMPRESSION: 1. No hydronephrosis or nephroureterolithiasis. 2. No acute inflammatory mass, bowel obstruction, or ascites. 3. Distal colonic diverticulosis without evidence of diverticulitis.	Kelly B Colleton, PA-C	07/10 1606
WBC: 6.0	(Reviewed)	Kelly B Colleton, PA-C	07/10 1634
HGB: 14.0	(Reviewed)	Kelly B Colleton, PA-C	07/10 1634
	On independent review of the patient's UA from outside facility there was only trace amount of blood no evidence of any other infectious process such as bacteria, nitrites or leukocytes.	Kelly B Colleton, PA-C	07/10 1635
Sodium: 141	(Reviewed)	Kelly B Colleton, PA-C	07/10 1649
Potassium: 3.7	(Reviewed)	Kelly B Colleton, PA-C	07/10 1649
Chloride: 106	(Reviewed)	Kelly B Colleton, PA-C	07/10 1649
Carbon Dioxide: 28	(Reviewed)	Kelly B Colleton, PA-C	07/10

ED Course (continued)

Value	Comment	By	Time
		C	1649
. Alkaline Phosphatase: 81	(Reviewed)	Kelly B Colleton, PA-C	07/10 1649
. AST: 19	(Reviewed)	Kelly B Colleton, PA-C	07/10 1649
. ALT: 16	(Reviewed)	Kelly B Colleton, PA-C	07/10 1649
. BUN: 18	(Reviewed)	Kelly B Colleton, PA-C	07/10 1649
. Creatinine: 0.93	(Reviewed)	Kelly B Colleton, PA-C	07/10 1649
. Calcium: 9.1	(Reviewed)	Kelly B Colleton, PA-C	07/10 1649
. Anion Gap: 7	(Reviewed)	Kelly B Colleton, PA-C	07/10 1649
. Bilirubin, Total: 0.4	(Reviewed)	Kelly B Colleton, PA-C	07/10 1649
.  Glucose: 188	(Reviewed)	Kelly B Colleton, PA-C	07/10 1649

Dictations

None

ED Diagnosis

Diagnosis	Description	Comment	Associated Orders
Final diagnosis			
Acute left-sided low back pain with left-sided sciatica	Acute left-sided low back pain with left-sided sciatica	--	AMB REFERRAL TO PHYSICAL THERAPY

ED Disposition

ED Disposition	Condition	Comment
Discharge	--	--

Results

None

Imaging Results

CT Abdomen Pelvis WO IV Cont (Final result)

Result time 07/10/26 15:54:16

Final result by Scott W Thalman, MD (07/10/26 15:54:16)**Impression:**

1. No hydronephrosis or nephroureterolithiasis.
2. No acute inflammatory mass, bowel obstruction, or ascites.
3. Distal colonic diverticulosis without evidence of diverticulitis.

Electronically Signed by: Scott Thalman 7/10/2026 3:54 PM

Narrative:

EXAMINATION: CT ABDOMEN PELVIS WO IV CONT

INDICATION: 63 years old, Female. Possible ureterolithiasis

Imaging Results (continued)

COMPARISON: None

TECHNIQUE: Multiple axial helical sections from the level of the hemidiaphragms to the lesser trochanters without intravenous contrast. All CT scans at this facility use dose modulation, iterative reconstruction, and/or size-based dosing when appropriate to reduce radiation dose to as low as reasonably achievable.

The patient has had 0 other CT scans within the last 12 months.

FINDINGS:

Lung bases: Visualized lung bases are clear.

Heart: Visualized heart unremarkable.

Liver: Left hepatic lobe cysts and additional hypodensities that are too small to characterize, though likely additional benign cyst.

Gallbladder: Unremarkable.

Bile ducts: Unremarkable.

Pancreas: Normal.

Spleen: Unremarkable.

Adrenal glands: Normal.

Kidneys: No hydronephrosis or nephroureterolithiasis. Kidneys are normal in size.

Vasculature: Unremarkable.

Lymph nodes: No pathologically enlarged abdominal or pelvic lymph nodes according to size criteria.

Bowel: Normal caliber bowel. Mild distal colonic diverticulosis without evidence of diverticulitis.

Intraperitoneal space: No free air or free fluid.

Bladder: Unremarkable.

Reproductive: Unremarkable.

Regional skeleton: No worrisome bone lesion.

Other: Unremarkable.

ECG Results

None

Home Medications

	Taking?	Last Dose	Start Date	End Date	Provider
 cholecalciferol 2000 units tablet		--	--	--	Historical Orders
 cyclobenzaprine (FLEXERIL) 10 mg tablet	✓	--	04/26/25	--	Historical Orders
 hydrocortisone 2.5 % cream	✓	--	10/02/25	--	Historical Orders
 Mag Chloride-Calcium Carb (SLOW-MAG) 64 mg elemental		--	--	--	Historical Orders
 methocarbamol (ROBAXIN) 500 mg tablet	✓	--	07/06/26	--	Historical Orders
 Multiple Vitamins-Minerals (MULTI FOR HER) tablet		--	--	--	Historical Orders
 niacin 100 mg tablet		--	--	--	Historical Orders

Medication Administration from 07/10/2026 1459 to 07/10/2026 1731

None

Medication Administration from 07/10/2026 1459 to 07/10/2026 1731 (continued)**ED Prescriptions**

Medication	Sig	Dispense	Start Date	End Date	Auth. Provider
methylPREDNISolone (MEDROL, PAK,) 4 MG therapy pak tablet	follow package directions	21 tablet	7/10/2026	--	Kelly B Colleton, PA-C

ED Medication Orders (From admission, onward)

None

Code, Iso, Restraint (From admission, onward)

None

ED Imaging Orders (From admission, onward)

Start	Ordered		Status	Ordering Provider
07/10/26 1513	07/10/26 1512	CT Abdomen Pelvis WO IV Cont 1 time imaging	Final result	COLLETON, KELLY B

ED Micro, Lab, POCT (From admission, onward)

Start	Ordered		Status	Ordering Provider
07/10/26 1550	07/10/26 1549	CBC With Plt Count And Auto Diff STAT Collect/Perform - Lab	Final result	COLLETON, KELLY B
07/10/26 1550	07/10/26 1549	Comprehensive Metabolic Panel STAT Collect/Perform - Lab	Final result	COLLETON, KELLY B

ED All Orders (From admission, onward)

Start	Ordered		Status	Ordering Provider
07/10/26 1550	07/10/26 1549	CBC With Plt Count And Auto Diff STAT Collect/Perform - Lab	Final result	COLLETON, KELLY B
07/10/26 1550	07/10/26 1549	Comprehensive Metabolic Panel STAT Collect/Perform - Lab	Final result	COLLETON, KELLY B
07/10/26 1513	07/10/26 1512	CT Abdomen Pelvis WO IV Cont 1 time imaging	Final result	COLLETON, KELLY B
07/10/26 0000	07/10/26 1703	AMB Referral to Physical Therapy Specialty: PHYSICAL THERAPY Provider: (Not yet assigned)	Ordered	COLLETON, KELLY B

Discharge Orders (From admission, onward)

Start	Ordered		Status	Ordering User
07/10/26 0000	07/10/26 1703	AMB Referral to Physical Therapy Specialty: PHYSICAL THERAPY Provider: (Not yet assigned)	Ordered	COLLETON, KELLY
07/10/26 0000	07/10/26 1706	methylPREDNISolone (MEDROL, PAK,) 4 MG therapy pak tablet	Ordered	COLLETON, KELLY

Allergies (Reviewed on: 07/10/26)

Agent	Severity	Comments
Warfarin		

Tetanus Up To Date

None

Medical History

No past medical history on file.

Surgical History

No past surgical history on file.

Social History

Tobacco History

Smoking Status

Never

Smokeless Tobacco Use

Never

Alcohol History

Alcohol Use Status

Not Currently

Comment

socially

Drug Use

Drug Use Status

Never

Sexual Activity

Sexually Active

Not Asked

Other Factors

Not Asked

Social Documentation

No social documentation on file.

Family Medical History

None

ED LDA Documentation (7/9/2026 00:00 to 7/10/2026 17:31)

ED Events

Date/Time	Event	User	Comments
07/10/26 1459	Patient arrived in ED	KLUDY, TERYN	--
07/10/26 1459	Emergency encounter created	KLUDY, TERYN	--
07/10/26 1519	Triage Started	TYLER, NICHOLE	--
07/10/26 1520	Triage Completed	TYLER, NICHOLE	--
07/10/26 1548	Patient roomed in ED	DOWNS, EMMA	To room VC
07/10/26 1549	Assign Attending	COLLETON, KELLY	Kelly B Colleton, PA-C assigned as Attending
07/10/26 1549	Assign Physician	COLLETON, KELLY	--
07/10/26 1551	Registration Started	VALDEZ, NADIA	--
07/10/26 1551	Assign Nurse	DOWNS, EMMA	Emma Downs, RN assigned as Registered Nurse
07/10/26 1613	Registration Completed	VALDEZ, NADIA	--
07/10/26 1731	ED Tracking End	DOWNS, EMMA	--
07/10/26 1731	Patient discharged	DOWNS, EMMA	--

Follow-up Information

None

Discharge Instructions

You were seen in the emergency department for concerns of left-sided flank and low back pain. They did have some concerns that this could be related to a kidney stone but your CT showed no signs of this. Your lab work was overall reassuring. Your exam is most consistent with sciatica. For this it can take some time to improve but the best care is with physical therapy therefore a referral has been sent. Recommend you continue to use the muscle relaxers and Tylenol as needed for pain or discomfort. You can also apply lidocaine patches. Sent a prescription for a steroid pack which can sometimes help with the inflammation. Take as prescribed or instructed on the box.

If you have any additional concerns or persistent discomfort recommend follow-up with your PCP.

Return to the Emergency Department if you experience worsening back pain, difficulty walking, fevers, numbness, tingling, incontinence, or any other concerning symptoms.

Discharge References/Attachments

None

AVS Reports

Date/Time	Report	Action	User
7/10/2026 5:07 PM	ED AVS (After Visit Summary)	Printed	Kelly B Colleton, PA-C

Communication Routing History

None

ED Charges

Description	Code	Date	Service Prov	Modifiers	Qty	Status
HB EMERGENCY DEPT VISIT 4	6000008	07/10/2026	Downs, Emma	25	1	Filed
HB COMPREHENSIVE METABOLIC PANEL	3100145	07/10/2026	Colleton, Kelly B		1	Filed
HB CBC WITH PLT COUNT AND AUTO DIFF	3100386	07/10/2026	Colleton, Kelly B		1	Filed
HB VENIPUNCTURE (NON ARUP)	3000002	07/10/2026	Colleton, Kelly B		1	Filed